A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

22 April 2026

Dr Jeremy Chan
Tyner Road Medical Centre

Dear Dr Chan,

Re: Mr Bradley Thomas
DOB: 1976-11-29
Address: 57 Underwood Road, BORONIA, VIC, 3155

Summary

- **Gastroscopy and colonoscopy revealing healthy mucosa and no significant pathology.**
- **Excision of a small, benign colonic polyp with no immediate clinical significance.**
- **Significant reduction in frequency of emesis on a maintenance dose of nortriptyline.**
- **Surveillance colonoscopy recommended in five to ten years.**
- **No action required by you.**

I had the pleasure of conducting a review with Bradley Thomas following his recent gastroscopy and colonoscopy, on a background of his small bowel atresia, prior resections and concomitant IBS. You will be pleased to note that the procedures were uncomplicated and the findings were very reassuring.

The bowel preparation was of high quality, which allowed for a thorough examination of the colonic mucosa. A small, diminutive polyp was identified and removed; histologically, this was found to be insignificant with no malignant potential. Multiple biopsies were also taken from the stomach, small intestine, and oesophagus. These have excluded any evidence of infection, autoimmune conditions, or other significant gastrointestinal pathology.

Clinically, Bradley reports a marked improvement in his symptoms compared to his baseline in 2018–2020. At that time, he was experiencing episodes of emesis several times per week. Currently, these episodes occur only once every one to two months. He acknowledged some inconsistency with his medication compliance, noting that when he ceases his nortriptyline

All appointments and enquires
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East Melbourne VIC 3002
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Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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(Allegron), his symptoms tend to recur within a few weeks, particularly when exacerbated by work-related stress or anxiety.

Bradley is currently focused on lifestyle modifications, including weight management and exercise. He is also self-monitoring his blood pressure, as he noted a recent elevation during his perioperative assessment. He has previously been on antihypertensive therapy but is currently attempting to manage this through stress reduction and lifestyle changes.

Impression and Plan

The overall clinical picture is one of significant improvement. I have reinforced the importance of medication adherence, as the nortriptyline appears to effectively blunt the impact of psychological stressors on his gastrointestinal function. I have provided him with a new prescription for Allegron to ensure continuity of treatment. From a procedural standpoint, a surveillance colonoscopy is not required for another five to ten years. I have arranged to review him in 12 months' time to monitor his progress, though he has been advised to contact my rooms sooner should his symptoms deteriorate.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

THOMAS, BRADLEY JAMES : SVP 712078

DOB: 29/11/1976 Age: 49 years

57 Underwood Rd, Boronia 3155

Results from 11/04/2026 ..... Episode# 26P275614 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

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Lab #: 2026B009423 **CLINICAL NOTES** Hx of GERD, PHx of small bowel atresia. Gastric = pylorus.? IM. Duo = N. Oesophagus =mix: Lower, mid, upper -? ##Illegible## inlet patch?. Random colon. Rectal polyp = HP. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "lower Oesop". Four mucosal fragments measuring 1-5 mm. A1. 2. Specimen labelled "Gas". Two mucosal fragments measuring 3-7 mm. A1. 3. Specimen labelled "Duodenum". Two mucosal fragments measuring 2-3 mm. A1. 4. Specimen labelled "Random Colon". Four mucosal fragments measuring 1-4 mm. A1. 5. Specimen labelled "Rectal Polyp". Two tan polypoid fragments 2 x 1 x 1 mm to 3 x 2 x 1 mm. Processed whole. A1. **MICROSCOPIC DESCRIPTION And DIAGNOSIS** **1. Lower oesophagus - Four (4) strips of squamous mucosal tissue with no significant pathological features. No eosinophils are seen. No fungal elements are seen. No glandular mucosal tissue is seen. No evidence of dysplasia or malignancy. 2. Gas - Morphological features of chronic inactive gastritis of chemical/reactive type with focal complete intestinal metaplasia. No Helicobacter pylori-like organisms are seen. No evidence of dysplasia or malignancy. 3. Duodenum - Duodenal mucosal biopsies with no significant pathological features. Morphological features of coeliac disease are not seen. No granulomas or parasites are identified. No evidence of dysplasia or malignancy. 4. Random colon - Large bowel mucosal biopsies with no significant pathological features. There is no ulceration, architectural distortion or "bottom heavy" inflammation. No granulomas are seen. There is no evidence of microscopic colitis. No parasites are seen. There is no evidence of dysplasia or malignancy. 5. Rectal polyp - Features consistent with an hyperplastic polyp. No evidence of dysplasia or malignancy.** Pathologist: Dr Cesar Salinas-LaRosa

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